

Public Health Emergency Operations Center
"COUSP DRC"

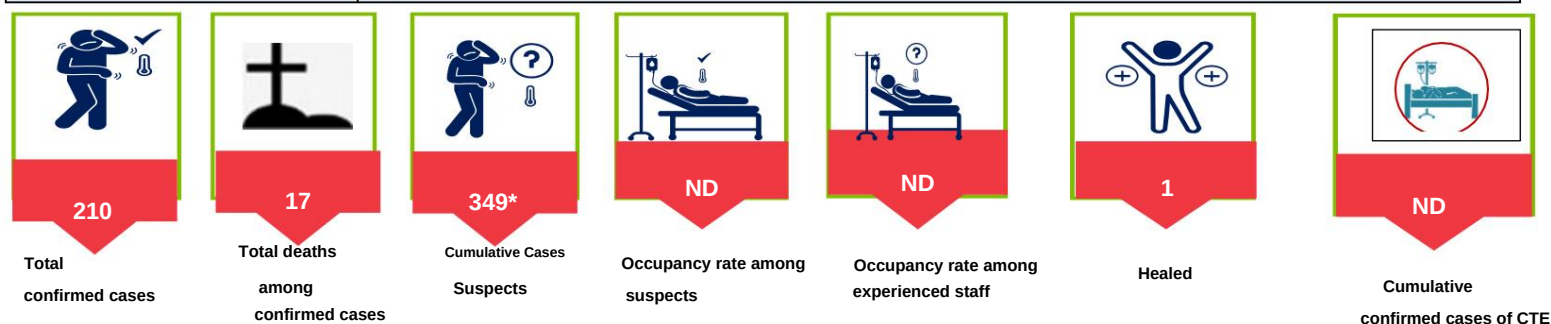
MVE-17 Incident Management System

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Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°014/MVB_28/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (13)	ITURI : Aru, Damascus, Bunia, Kilo, Mongbwalu, Nizi, Nyankunde and Rwampara NORTH KIVU : Butembo, Goma, Oicha, Kalunguta and Katwa SOUTH KIVU : Miti-Murhesa
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- * The number of suspected cases has been revised downwards, as these have been investigated and samples taken, the results of which have confirmed some cases and refuted others.
- * Suspected deaths have been temporarily excluded from the count pending the results of ongoing investigations, which will either confirm them and classify them as probable cases, or rule them out permanently.

0. HIGHLIGHTS

- Arrival of the Minister of Health in Bunia on May 28, 2026, accompanied by the Director General of the INSP and the coordinator of the COUSP as well as the delegation of 37 multidisciplinary technical experts at the national level.
- 84 New confirmed cases as of May 28, 2026, including 5 PPL in the Bunia health zone.
- One (1) new health zone affected in the province of Ituri, namely that of Damas.

I. CONTEXT

The Ituri province is located in the northeast of the Democratic Republic of Congo and shares a long border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. It has an estimated population of over 8 million, including more than one million internally displaced persons.

In September 2018, this province was affected by an Ebola virus disease epidemic that was raging in the neighbouring province of North Kivu.

The Mongbwalu health zone is one of the 36 health zones in Ituri province. Considered the starting point of the epidemic, it is located in the Djugu territory, where several armed groups operate. It is situated 70 km from Bunia, the capital of Ituri province, and experiences frequent population movements towards Uganda. This area is characterized by numerous construction sites that attract people from other territories within the province and from neighboring provinces.

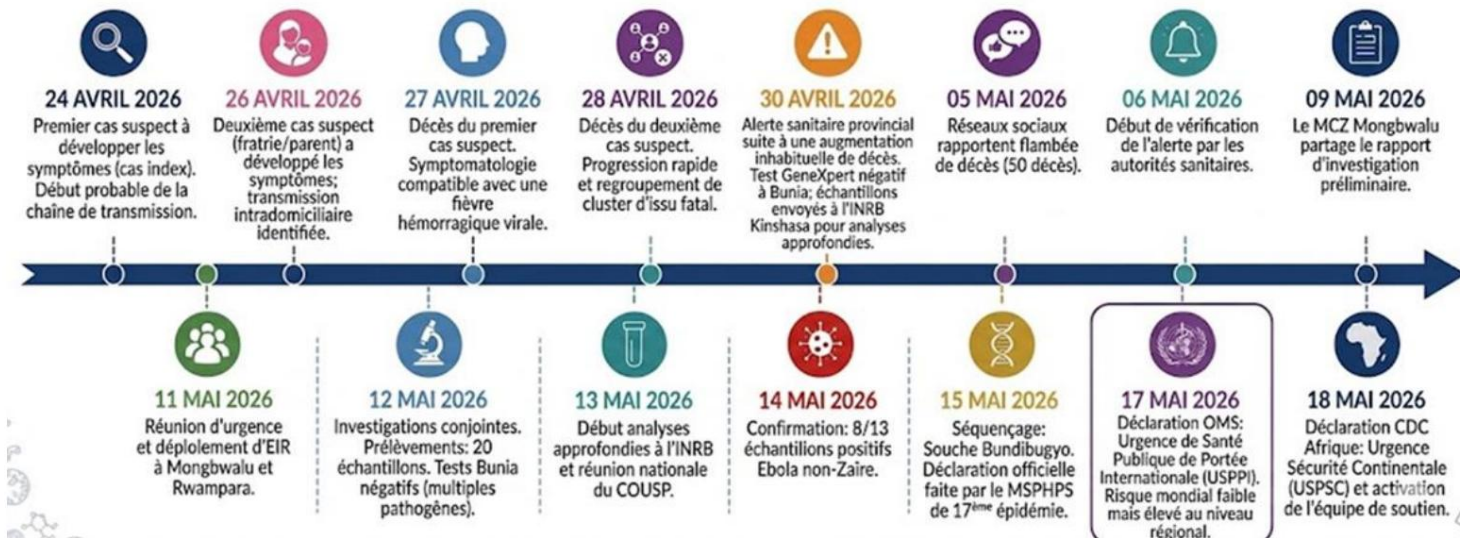
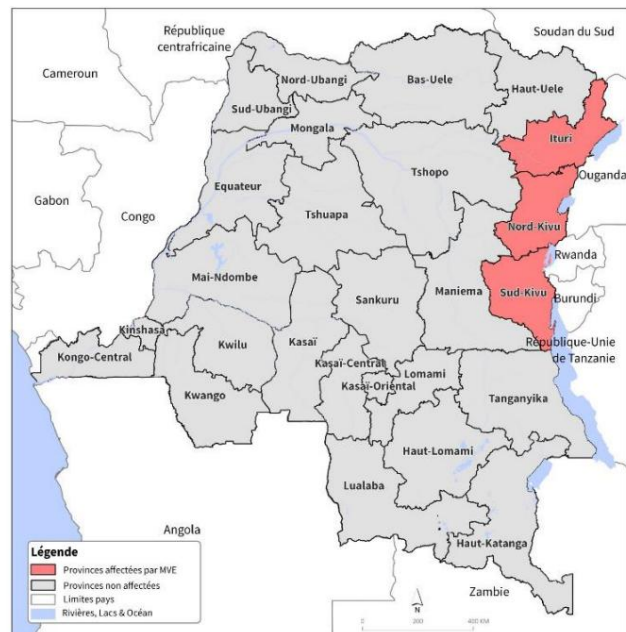
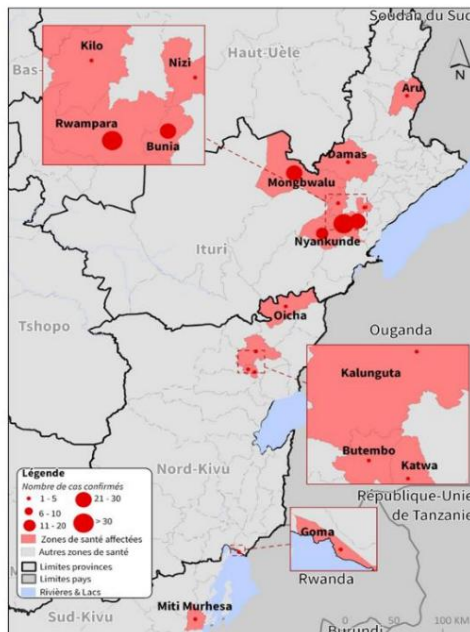


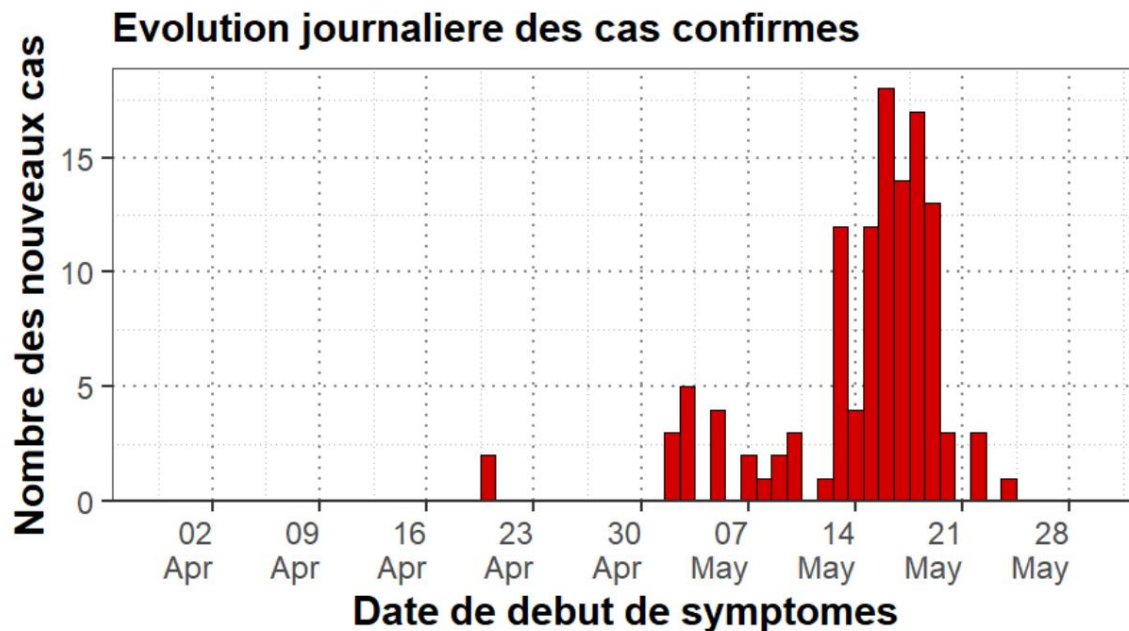
Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.



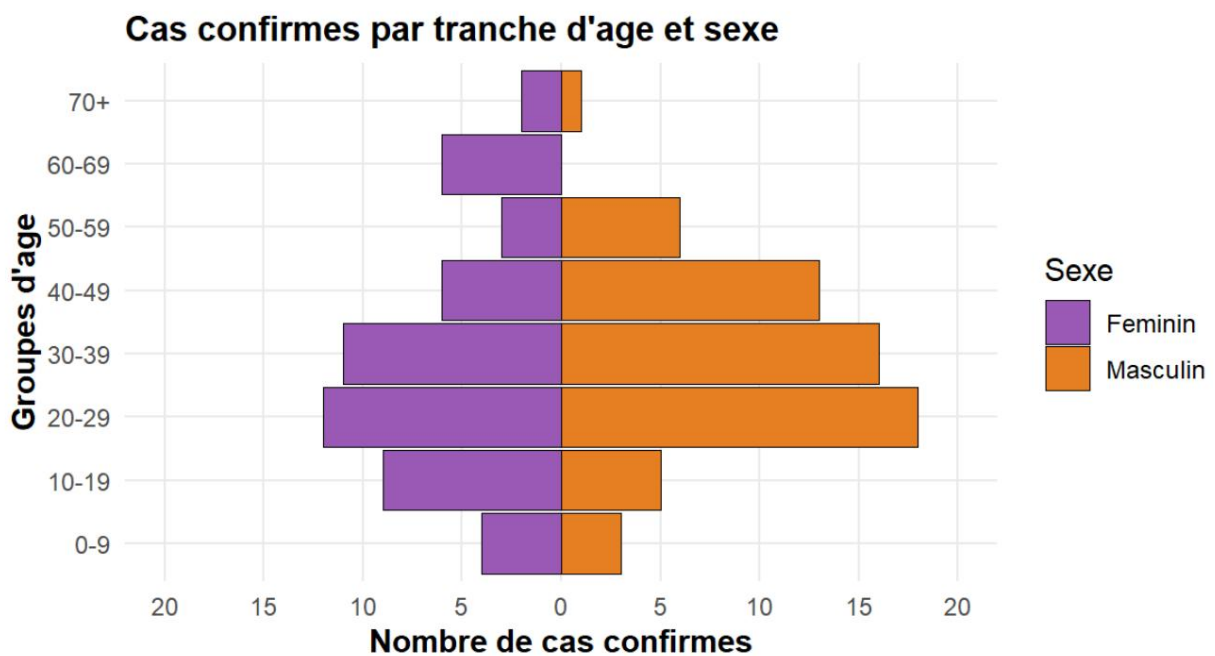
Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of May 28, 2026.

II. EPIDEMIOLOGICAL SITUATION UPDATE

- (84) new confirmed cases were notified for the day of May 28, 2026.
- Thus, the cumulative number of confirmed cases as of May 28, 2026: 210 cases for the three affected provinces (Ituri, North Kivu and South Kivu).
- Three provinces are currently affected, namely Ituri, North Kivu and South Kivu.
- A new ZS affected in Ituri, namely that of Damas, bringing the number of affected ZS to 8 in Ituri and 15 across the country.
- ITURI: epicenter with 8/36 affected health zones (Aru, Bunia, Kilo, Rwampara, Mongbwalu, Nizi, Nyankunde and Damascus)
- NORTH KIVU: 6/35 ZS affected (Goma, Butembo, Oicha, Kalunguta, Katwa and Kyondo)
- SOUTH KIVU: 1/34 ZS affected (Miti-Murhesa)



Riposte MVE, mai 2026. Donnees inclues dans l'analyse : n cas confirmes = 120.



acker - Riposte MVE, mai 2026. Donnees inclues dans l'analyse : n cas confirmes = 115.

Comments : The 20-49 age group is the most affected, with a predominance of males.

III. RESPONSE ACTIONS NSE

IV.1. COORDINATION

- Holding a tripartite press briefing between the Provincial Governor, the Minister of Health, and the INSP to inform the public about the government's commitment to ending this epidemic (making available the necessary resources and mobilizing partners).
- Mission to assess the health response by the Ministers of Health and Communication while reaffirming the Government's commitment to the affected populations.
- Interview with the Deputy Incident Manager with 10 local media on ongoing response operations, with a focus on the implementation of Safe and Dignified Burials (SDB).



IV.2. EPIDEMIOLOGICAL SURVEILLANCE

- Continuation of field surveillance activities (reporting alerts, investigations, active search, listing and monitoring contacts, retrospective documentary review of cases and deaths, etc.).
- Coordination meeting with the monitoring pillar for guidance on urgent actions.
- Support for affected health zones: data harmonization and coaching on alert management in the Bunia Special Zone.
- For May 28, 2026, 99 alerts were reported, 75 (75.7%) were investigated, and 67 (89.3%) have been validated, of which 54 (80.6%) were taken (partial data).



Point of Entry and Point of Control (PoE/PoC) Surveillance:

- Guided tour of the arrival and departure areas of the airport for passengers, led by Their Excellencies the Minister of Health and the Minister of Communication.
- Water supply to PoCs, installation of 3000 L water storage devices in the 3 PoCs and connection of all handwashing devices to the storage devices with the support of Samaritans Purse at the PoCs (Tchai, Dele and Mudzibala).

Table III. Summary of PoE/PoC data as of 28/05/2026

Indicators at PoE/PoC	TOTAL
Travelers who have used the PoE/PoC	16,905
Screened travelers	16704 (98.8%)
Travelers who have washed their hands	16.674 (98.6%)
Travelers made aware of Ebola virus disease	16,704 (98.8%)
Listed contacts	0
Alerts raised	2 (ZS Aru)

IV.3. LABORATORY

- Continued analysis at the Bunia laboratory and the INRB Kinshasa.

Table IV. Cumulative laboratory situation as of 28/05/2026

Key indicators	Total
Samples received	883
Re-sampling (control)	8
Number of samples analyzed	755
Positivity rate	27.8%
Total number of positive cases	210

IV.4. INFECTION PREVENTION & CONTROL

- Continued training of trainers and support partners on the complete PCI package (J2).

1. ZS Bunia

- Allocation and receipt of 45kg of chlorine to the PCI pillar.
- Decontamination of 2 households in Solenyama
- 3 EDS carried out on 3 alert cases (1 HGR-Bunia and 2 CS Aero).

2. ZS Rwamapara

- Briefing of 2 healthcare providers on bed spacing (Maternity) (CS Rwamapara).

3. ZS Nyakunde

- 2 EDS carried out on 2 alert cases and 2 decontaminations in the community.

4. ZS Aru

- 1 EDS carried out on 1 alert case.
- Disinfection of the Bakitha room.

5. ZS Mongwalu

- 7 EDS carried out on 9 alert cases (1 HGR-Mongwalu, 1 AS Lodjo, 4 AS Mongwalu, 2 AS Ceca 20).



Decontamination of the Lembabo Hospital
Bunia, Ituri province.

Allocation of EDS inputs to BCZ
Bunia and Rwamapara

IV.5. HOLISTIC CARE

- Briefing of service providers on nutritional status assessment.
- Handing over of MUACs for nutritional activities.
- Field visit to monitor the work of the CTEs.

• Psychosocial interventions and mental health:

- (i) Announcement: 4 deaths followed by psychological support for families regarding the EDS;
- (ii) Briefing of clinical psychologists on the announcement of confirmed cases and deaths;

- **Continuity of care:** 2 teams of evaluators were briefed on the assessment of ESS in the area health of RWAMPARA;

Table V. Patient movement in healthcare facilities as of 28/05/2026

Indicators and Variables		HGR BUNIA	HGR RWAMPARA	CME	HGR MUNGWALU	HGR NYAKUNDE	CT BAMBU	CT HEAL AFRICA (North Kivu) 19 beds	TOTAL and Occupancy rate
		(12 Beds: 08 S and 03 C)	(18 Beds: 8 C and 10 S)	(27 Beds: 08 Conf and 19 S)	(40 beds: 12 C and 28 S)				
Report		14	8	22	10	39	6	2	101
News	PPL	0	0	0	1	0	0	0	1
Admissions	Others	2	2	5	1	0	3	0	13
Outings	Healed	0	0	0	0	0	0	0	0
	Deceased	1	1	1	2	1	0	0	6
	No Case	0	0	0	0	0	0	0	0
	Escapees	3	0	0	1	0	0	0	4
	Transferred	0	0	0	0	0	0	0	0
Patients Hospitalized	Total	12	9	26	9	38	9	0	103
	New Cases Confirmed	0	1	0	0	0	0	0	1
	Ancients Case Confirmed	1	0	0	0	9	1	1	12
	Suspects	11	8	26	9	29	8	1	92
	Suspect < 5 years	0	0	0	0	0	0	0	0
Bed occupancy rate	Suspects	11/12 (91.6%)	8/10 (80%)	26/27 (96%)	9/28 (32%)	ND	ND	ND	70.1%
	Confirmed	1/3 (33.3%)	1/8 (12%)		0/12 (0%)	ND	ND	ND	8.7%

S: Suspects C: Confirmed ND: Not available

IV.6. CREC

Managing rumors surrounding EDS, decontamination and the decrease in attendance at ESS.

- 21 journalists from local media outlets in Bunia (radio, TV and online media) have pledged to communicate only information

- Commitment of 31 Community Action Leaders (mayor and heads of various services) from the SHARI

Commune (Bunia Health Zone) to prevent and manage the infodemic and community resistance **in the Bunia Health Zone:** 478 people sensitized at the Bunia central market on preventive measures against Ebola Virus Disease (EVD), including 323 women and 135 men. Deployment of communication materials in strategic locations in Bunia (MURONGO National Airport and Bunia Central Market).

ZS RWAMPARA: Awareness-raising of two traditional healers who treated two patients who died in AS Shari and preparation for a visit with the Surveillance pillar to further investigations.

Local communication

ZS Bunia and

Rwampara: • 7,323 people reached by awareness messages during the VADs carried out by Red Cross volunteers: 1,024 men, 1,286 women, 2,626 girls, 2,327 boys.

ZS Bunia:

- 3,662 students, including 2,414 girls and 1,248 boys, were made aware of preventive measures in 5 schools. of the MVE.

ZS Nizi: Educational talks with 90 RECOS including 30 in AS LINGO, 30 in AS LOPA and 30 in AS LOPA Mission on preventive measures for Ebola virus disease and the referral of patients to the nearest healthcare facilities.

IV.7. LOGISTICS

- Supply of PCI inputs and materials to
Category 1 ESS
- Monitoring the progress of the construction work
CTE at the BUNIA General Hospital;
- Monitoring the installation work of 3 tents at
CME/BUNIA.
- Visit to the hydraulic and electrical installations at
Provincial laboratory.



IV.8. SECURITY

The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm.

- Security briefing for newly deployed teams.

IV. CHALLENGES

- Reluctance towards post-mortem sampling: Formal refusal by families to have diagnostic tests (swabs) carried out on the bodies of the deceased in morgues.
- Absence of standardized Ebola Transit Centre and Treatment Centre in Ituri.
- Weakness in Contact Follow-up and Lack of Training for Stakeholders
- Persistence of gaps in the identification and rigorous monitoring of contact cases at the level of active and high-risk health zones.
- Shortages in isolation facilities: Total absence of essential medications at the Center Ebola treatment (ETC) and at the isolation block in Rwampara and Nyankunde.
- Supply failure of reference hospitals: Lack of supply of Essential Generic Medicines (EGMs) within the four targeted General Reference Hospitals (GRHs), weakening the overall care of patients.
- Lack of IPC equipment (PPE, DLM, chlorine) in North Kivu
- Low reporting of alerts by zones in the 3 affected provinces (Ituri, North Kivu and South Kivu) Kivu);
- Insufficient financial resources to support all pillars of the response in the health zones affected;
- Difficulty in transporting samples to laboratories in Goma and Kinshasa.
- Spread of rumors and proliferation of traditional treatment recipes.

- Lack of communication materials and tools.

V. PERSPECTIVES

- Training of service providers on the use of PCR RADIONE
- Training of service providers on Ebola virus disease (EVD) and squamous cell carcinoma (SCC) surveillance in affected and high-risk health zones risk of three provinces being affected;
- Training of community health workers on Ebola virus disease and SBC surveillance in affected health zones and high risk in the province of ITURI.
- Finalize the development of the research protocol and the data collection tools for perception communities on Ebola, the response and actors in health zones affected by the Ebola epidemic.



Preparation for implementation at the EDS in the Bunia health zone, Ituri province.



Day 2 of the training of trainers and support partners on the complete PCI package

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